

KIIRA MOTORS CORPORATION

HEALTH AND WELLNESS MANAGEMENT SYSTEM

SOFTWARE REQUIREMENTS SPECIFICATION

Field	Detail
Document name	Software requirements specification
Reference number	KMC.DQHSE.08/26-SP001
Document description	Requirements for the Health and Wellness Management System covering the patient registry, clinic visits, laboratory requisitions, medical referrals and divisional performance reporting
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1 Introduction

1.1 Purpose

This Software Requirements Specification defines what the Health and Wellness Management System must do. It is written for the Health and Wellness Division who will use the system, for the Department of Quality, Health, Safety and Environment who own it, and for KMC ICT who will host and later maintain it.

The Division originally asked for a dashboard into which staff would type summary figures at the end of each working day. This specification proposes something different, and the difference is the central idea of the system. A dashboard fed by end of day typing is a second manual process running alongside the first. It inherits every weakness of the paper it replaces, namely transcription error, delay, and no verifiable link between a reported figure and the event that produced it. The system therefore captures each operational event where it happens, and derives the dashboard from those records. The dashboard is a consequence of the system rather than a component of it.

1.2 Scope

The system covers the Health and Wellness division of the department. That division comprises three units, which are Occupational Health, Ergonomics and Wellness, and Industrial Hygiene. Occupational Health covers clinic attendance and laboratory testing.

Quality Inspection and Testing, Workplace Safety, and Environment and Sustainability are outside scope. The client established after the review of 4 August 2026 that those divisions already operate their own systems. Occupational health and safety indicators that the division itself reports remain in scope, because they appear on the divisional performance dashboard the client supplied.

1.3 Definitions

Table 1.1: Terms used in this specification

S/N	Term	Meaning
1	Officer	Health and Wellness Officer. The clinical role. The only role that may retrieve an individual clinical record
2	Manager	Health and Wellness manager. Reviews performance and administers user accounts. Holds no clinical access
3	Director	Executive consumer of the dashboard. Holds no clinical access and no entry rights
4	Visit	One attendance at the clinic by one patient. Called an encounter internally
5	Requisition	A laboratory request on form KMC.DQHSE.05/26-FM008, together with the results written against it
6	Referral	A referral to an external medical facility on form KMC.DQHSE.02/26-FM004
7	Section state	Whether a part of the visit record is not recorded, partial, complete, or not clinically indicated
8	Provenance	Whether a reported figure was derived from records, entered from an external source, or is a proposal awaiting approval

1.4 Sources

Every requirement in this document derives from one of four sources. Where a requirement derives from none of them it is marked as a proposal and named as such, so that a reader can tell what KMC asked for from what was suggested.

Table 1.2: Source documents and their status

S/N	Source	Contribution	Status
1	KVP Infirmary Medical Form	The structure and order of the clinic visit record	Supplied by the division
2	KMC.DQHSE.02/26-FM004	The referral to an external facility, field by field	Supplied by the division
3	KMC.DQHSE.05/26-FM008	The laboratory requisition and its seven investigations	Supplied by the division
4	Divisional performance dashboard	The indicator set, targets and directions	Supplied by the division
5	Requirements interviews and reviews	Roles, workflow and the decisions in Section 8	Recorded in the decision register

2 Users and access

2.1 User roles

The system has three roles. The separation between them is the most consequential design decision in this specification and it is treated as a requirement rather than a configuration choice.

Table 2.1: Roles and what each may do

S/N	Role	May see	May enter	Clinical access
1	Health and Wellness Officer	Everything except account administration and reference changes	Patients, visits, laboratory, referrals, monitoring, occupational plans and monthly returns	Yes, and each retrieval is logged
2	Manager	Dashboard, unit pages, monthly returns, reference data and accounts	Accounts, KPI targets and exposure limits through the API	None
3	Director	Dashboard and summary reports	Nothing	None

The Director sees less than the Manager. That reflects the oversight task rather than lesser authority, and it is recorded here because it inverts the usual hierarchy and would otherwise be read as a defect.

2.2 The clinical access rule

An individual clinical record is accessible to the Health and Wellness Officer alone. No other role may retrieve one. Not the Manager, not the Director, not a system administrator, not a report generator, and not a background job.

This is the standard occupational health separation and it protects three parties at once. It protects the employee from employment consequences flowing from a medical disclosure. It protects the officer

from pressure to disclose. It protects the Corporation from liability under the Data Protection and Privacy Act, 2019, under which data concerning health is special personal data.

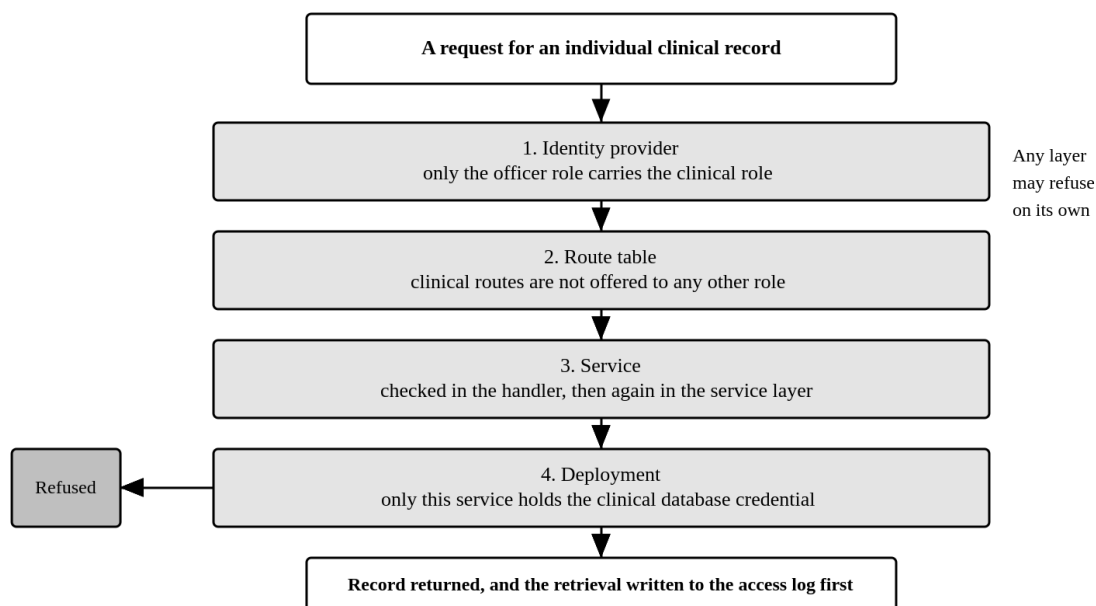


Figure 2.1: Where the clinical access rule is enforced

The arrangement in Figure 2.1 is deliberate repetition. Any one of the four layers refuses the request on its own, so no single mistake in configuration, routing or code removes the protection.

Table 2.2: Access requirements

S/N	Reference	Requirement
1	ACC-01	Every user is authenticated before any function is available
2	ACC-02	Only the officer role may retrieve an individual clinical record
3	ACC-03	Authorisation is enforced by the service and not only by hiding controls in the interface
4	ACC-04	Every retrieval of an individual clinical record is written to an access log before the record is returned
5	ACC-05	The access log is append only and readable by the data protection owner alone
6	ACC-06	Every creation, change and deletion of a user account is written to the audit log
7	ACC-07	The manager creates officer and director accounts. The manager cannot grant the manager role
8	ACC-08	A session ends after a period of inactivity, set conservatively because clinic workstations are shared
9	ACC-09	An account holding no role is refused at sign in and told who assigns roles

3 Patient registry

The registry identifies any person eligible to attend the clinic, whether or not they are an employee. It is keyed on a patient identifier and not on a staff number, because a registry that demanded a staff number would force clinical staff either to turn a patient away or to invent an identifier under time pressure. Neither is acceptable.

Table 3.1: Patient registry requirements

S/N	Reference	Requirement
1	PAT-01	A patient is registered with name, age, sex, phone contact, department, division, unit and job title
2	PAT-02	A patient carries a category of employee, intern or other
3	PAT-03	The staff number is optional for every category
4	PAT-04	Incomplete organisational detail does not prevent registration, because urgent presentation must never be blocked by data entry
5	PAT-05	The registry is searchable by name, staff number and department
6	PAT-06	Searching the registry is itself recorded in the access log, because a search reveals who attends
7	PAT-07	A patient record is removed by soft deletion only

4 Clinic visits

4.1 Structure of the record

The visit record follows the order of the paper form the clinic already uses. Clinical staff have muscle memory for that sequence and reordering it for interface convenience would cost more than it saves.

The full form is rarely completed. A patient presenting with a minor complaint will have no systemic examination and no occupational history recorded, and that is a clinical judgement rather than an omission. Each section therefore carries an explicit state.

Table 4.1: Section states and what each means

S/N	State	Meaning
1	Not recorded	Nothing has been entered and no decision has been taken about the section
2	Partial	Entry has begun and is not finished
3	Complete	The section has been completed
4	Not clinically indicated	The officer has decided the section does not apply to this visit

The distinction between not recorded and not clinically indicated is the whole purpose of holding a section state. A blank field on paper cannot tell the two apart, and recording the difference is what makes a partially completed record auditable rather than merely tolerated.

4.2 Visit requirements

Table 4.2: Clinic visit requirements

S/N	Reference	Requirement
1	VIS-01	A visit records the patient, date, time in and a visit type of walk in, referred by supervisor, emergency or follow up
2	VIS-02	The record is divided into the twelve sections of the paper form, in the order of that form
3	VIS-03	Each section carries one of the four states in Table 4.1
4	VIS-04	The number of sections carrying a clinical decision is shown against the visit
5	VIS-05	Whether the condition is work related is recorded as yes, no or unsure
6	VIS-06	Vital signs are recorded with their units, and body mass index is derived rather than entered

S/N	Reference	Requirement
7	VIS-07	A vital sign outside the usual range produces a warning and is recorded as entered
8	VIS-08	Entry in one section is not lost when the officer moves to another
9	VIS-09	Signing locks the visit. A signed visit cannot be edited
10	VIS-10	A correction to a signed visit is an appended amendment that preserves the original
11	VIS-11	A minimal walk in visit is recordable in under two minutes
12	VIS-12	The system records what the officer decided. It does not diagnose, recommend treatment or suggest a clinical action

Requirement VIS-07 deserves its reasoning. A genuinely abnormal reading is exactly the reading that matters clinically, and a system that refuses to record it is worse than paper. The interface warns and the service accepts.

5 Laboratory

The laboratory requisition reproduces form KMC.DQHSE.05/26-FM008. The clinic offers seven investigations in four groups. An earlier draft of this system carried a generic occupational health panel including liver function, lipid profile, audiometry and spirometry. None of that appears on the form the clinic uses, and it has been removed.

Table 5.1: Investigations offered on the requisition form

S/N	Group	Investigation	Note
1	Malaria and parasitology	Blood smear for malaria parasites	
2	Malaria and parasitology	Malaria rapid diagnostic test	
3	Gastrointestinal and serology	Typhoid antigen test	
4	Gastrointestinal and serology	Helicobacter pylori stool antigen	
5	Hematology	Complete blood count	
6	Blood glucose monitoring	Random blood sugar	
7	Blood glucose monitoring	Fasting blood sugar	Requires 8 to 12 hours of fasting

Two properties of the form drive the design and both differ from what a laboratory module is usually assumed to do. The results column sits beside the requested investigations column, so a result belongs to a requested investigation on one sheet rather than to a separate analyte record. The form carries no units and no reference ranges, so a result is recorded exactly as the laboratory reported it.

There is therefore no automatic abnormality flag. Deciding that a value is abnormal requires a reference range that KMC has not defined, and inventing one would place a clinical judgement in the software. Whether the laboratory wishes to supply structured ranges is recorded as an open decision in Section 8 rather than answered here.

Table 5.2: Laboratory requirements

S/N	Reference	Requirement
1	LAB-01	A requisition is raised from a visit, and patient information is carried across rather than typed again

S/N	Reference	Requirement
2	LAB-02	Patient information on the requisition is a snapshot taken when the requisition is raised
3	LAB-03	The officer selects one or more of the seven investigations printed on the form
4	LAB-04	The fasting instruction printed against fasting blood sugar is shown wherever that investigation appears
5	LAB-05	The requisition records a clinical summary and the authorising officer
6	LAB-06	The laboratory records the specimen collected, who collected it and the time of collection
7	LAB-07	A result is recorded against the investigation it was requested for, as written by the laboratory
8	LAB-08	The status of a requisition follows what has been recorded, so it cannot disagree with the form
9	LAB-09	A requisition is downloadable as a document reproducing the printed form
10	LAB-10	An investigation not printed on the form is refused

6 Referrals

The referral reproduces form KMC.DQHSE.02/26-FM004. It is implemented in the clinical service and in the signed-in interface. A referral is raised from a visit, passes through infirmery clearance and authorisation, is issued to the receiving facility, and is closed when the facility responds and the clinic reviews that response.

Patient identity and vital signs are derived by the service from a database join between the patient and visit. The join also proves that the visit belongs to the stated patient. This prevents a stale browser tab from filing one person's identity and vital signs against another person's attendance.

Table 6.1: Referral requirements and implementation position

S/N	Reference	Requirement	Position at issue
1	REF-01	A referral is raised from a visit. Patient identity and vital signs are derived server side from records joined on the patient and visit	Built
2	REF-02	The referral records clinical features, findings, general examination and past medical history as the form sets them out	Built
3	REF-03	The referral records the occupational consideration and the reason for referral	Built
4	REF-04	The referral carries a lifecycle of drafted, authorised, issued, returned and reviewed, moving forward one state at a time	Built
5	REF-05	The receiving facility response records diagnosis, treatment, follow up and any recommended sick leave. A correction keeps the referral reviewed and increments its version	Built
6	REF-06	Recommended sick leave reaches health related absenteeism through a transactional outbox without carrying patient identity or clinical narrative	Built
7	REF-07	A referral is downloadable as a document reproducing the printed form	Built in the browser
8	REF-08	The duplicate Section E labels, missing Section D and older role wording remain as printed until Document Control authorises a revision	Built, with DEC-025 and DEC-026 open

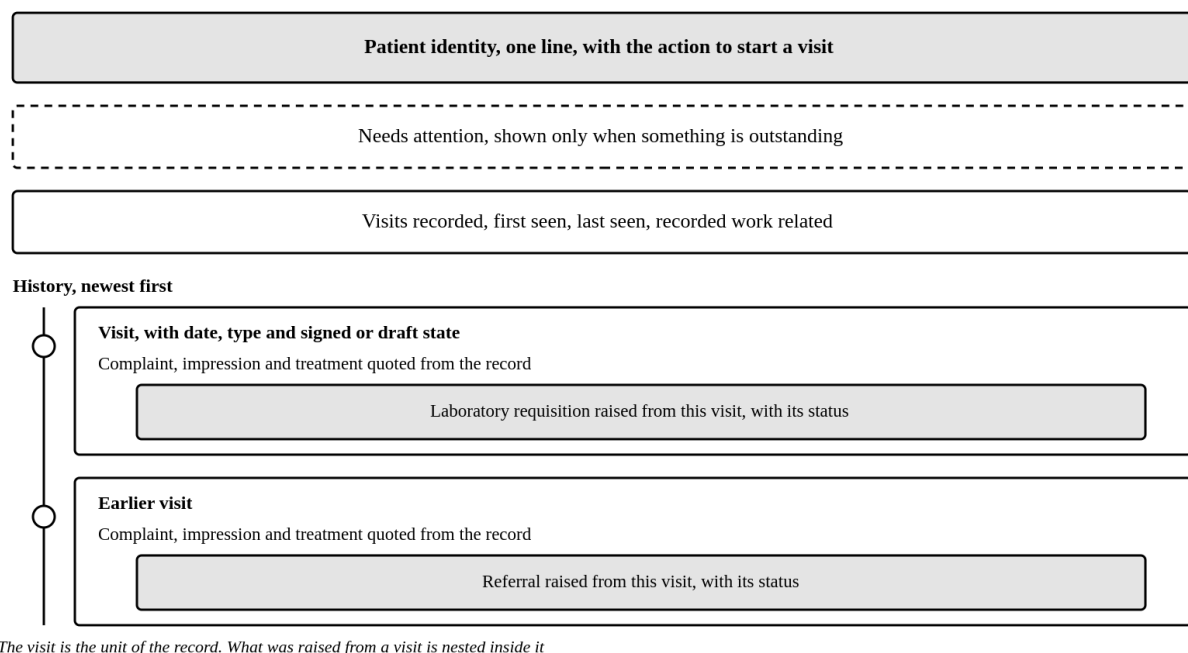
S/N	Reference	Requirement	Position at issue
9	REF-09	No management role may retrieve a referral or its authorisation summary until DEC-024 resolves the conflict between the form and the clinical access rule	Built as officer-only access

Requirement REF-06 matters more than it appears. Without it the referral is a document store rather than a part of the system, and the absenteeism figure loses a source it should have.

Section C still requires a decision. The printed form asks the Head of Division and the Chief of Staff to sign a document containing a provisional diagnosis, HIV status and mental health condition. That cannot stand alongside the rule in Section 2.2 that no management role sees an individual clinical record. The service can assemble a minimum-disclosure summary, but that route is officer-only and no management role can open referral content while DEC-024 remains unresolved.

7 Patient history

An officer meeting a patient asks one question before any other, which is what has happened to this person. Answering it from three separate registers, one for visits, one for laboratory and one for referrals, requires the officer to filter each and hold the join in their head. The records are already related, because a requisition and a referral are each raised from a visit, and presenting them as unrelated lists throws that relationship away at the moment it matters.



The visit is the unit of the record. What was raised from a visit is nested inside it

Figure 7.1: Organisation of the patient record

The organisation in Figure 7.1 takes the visit as the unit of the record, because that is how the clinical work actually happened.

Table 7.1: Patient history requirements and implementation position

S/N	Reference	Requirement	Position at issue
1	HIS-01	One patient record shows identity, what is outstanding, how much history exists, and the history itself	Built

S/N	Reference	Requirement	Position at issue
2	HIS-02	History is presented newest first, with one entry per visit	Built
3	HIS-03	Requisitions and referrals appear inside the visit they were raised from	Built
4	HIS-04	Each visit entry quotes the presenting complaint, the impression and the treatment	Built
5	HIS-05	Outstanding items are shown only when something is outstanding	Built
6	HIS-06	A requisition or referral whose visit is outside the loaded range is listed separately rather than omitted	Built
7	HIS-07	Opening a patient record writes one entry to the access log, not one per register consulted	Built
8	HIS-08	The officer may download a patient history summary containing signed visits only and a filename containing no patient name or identifier	Built in the browser
9	HIS-09	A patient history download creates a distinct export access event before the file is generated	Not built, DEC-037 is open

Requirement HIS-05 exists because a permanent banner reading that nothing is outstanding teaches people to stop reading banners, and the one occasion it matters is the occasion it is scrolled past.

8 Divisional performance

The operational registers and the dashboard are implemented. The dashboard presents the indicators the division reports for the selected month and for the year to date. Every figure states its provenance, because a figure that cannot be traced to a record or an attributed return cannot be defended when it is questioned.

8.1 Effective-dated reference data

KPI targets and industrial hygiene limits are held by the admin service as dated rows. A new value closes the row it replaces and opens another from its effective date. The target or limit itself is not overwritten, because doing so would make a historical report change when it is run again.

Table 8.1: Reference data requirements and implementation position

S/N	Reference	Requirement	Position at issue
1	ADM-01	A KPI target records its value, comparison, direction, approaching boundary, source, approval state and effective period	Built
2	ADM-02	An industrial hygiene limit records its parameter, context, value, unit, averaging period, standard, source, approval state and effective period	Built
3	ADM-03	Consumers resolve the row in force on the date being evaluated, rather than the row current today	Built
4	ADM-04	Superseding reference data closes the current row and creates a new dated row, so prior values remain available	Built
5	ADM-05	The authorised owner may change reference data without a code release	API built for the manager. Ownership and a screen remain open at DEC-035

8.2 Ergonomics and wellness

Ergonomic assessments and their corrective actions are implemented in the occupational service and in the interface. The dashboard measures follow-through rather than the assessment outcome, because finding a problem is not the same as closing the action raised from it.

Table 8.2: Ergonomics requirements and implementation position

S/N	Reference	Requirement	Position at issue
1	ERG-01	An assessment records its date, workstation, work type, assessor, outcome and findings	Built
2	ERG-02	The outcome is compliant, partially compliant or non-compliant, because the middle position is a distinct assessment result	Built
3	ERG-03	An assessment may create corrective actions with a description, owner and due date	Built
4	ERG-04	An action records open, implemented or closed status, and a closed action records its closure date and evidence	Built
5	ERG-05	Ergonomic risk control is actions closed on or before their due date divided by actions due in the reporting month	Built. Formula and target remain confirmation-pending
6	ERG-06	The officer writes the register and the manager reads it without write access	Built

8.3 Industrial hygiene

Monitoring events and readings are implemented in the occupational service and in the interface. Completeness is kept separate from compliance, because a unit must not appear more compliant merely by performing less monitoring.

Table 8.3: Industrial hygiene requirements and implementation position

S/N	Reference	Requirement	Position at issue
1	HYG-01	A monitoring event records the month, date, location, instrument, notes and whether it was performed	Built
2	HYG-02	An event not performed records a reason and cannot receive readings	Built
3	HYG-03	A reading records the parameter, value, monitoring context and the date on which it was taken	Built
4	HYG-04	The service resolves the limit in force on the reading date and copies the reference identifier, value, unit, averaging period and standard onto the reading	Built
5	HYG-05	The compliance result is written from that copied limit and a database trigger prevents later changes to the measurement, snapshot or result	Built
6	HYG-06	A reading outside the monitoring plan remains in the register but may be excluded from the KPI denominator	Built
7	HYG-07	The officer writes the register and the manager reads it without write access	Built

8.4 Monthly returns

The monthly occupational plan and the attributed return are implemented on one monthly returns page because the officer must see the operational counts, Human Resources figures, safety figures and linked referral leave together before reviewing the resulting rates. They remain separate records and each correction keeps its former value and the reason.

Table 8.4: Monthly return requirements and implementation position

S/N	Reference	Requirement	Position at issue
1	RET-01	The occupational plan records planned hygiene events, planned ergonomic assessments and confirmed occupational disease cases for the month	Built. Placement and ownership remain open at DEC-036
2	RET-02	The return records other health related lost days, month-end headcount, surveillance scheduled and completed, and the source of those figures	Built
3	RET-03	Fatalities, recordable incidents, recordable injuries and reportable near misses are optional attributed figures with a named source. Blank means No data	Built. Authoritative ownership remains open at DEC-027
4	RET-04	Correcting a plan or return requires a reason and keeps the prior row with who corrected it and when	Built
5	RET-05	The entry page computes absenteeism and surveillance rates while figures are typed, while the dashboard recomputes from stored inputs	Built
6	RET-06	Linked referral leave is shown separately and is not re-keyed into the return	Built
7	RET-07	No workplace safety incident register is provided while the authoritative system and owner are unresolved	Built as an intentional absence
8	RET-08	The officer writes the return and the manager reads it without write access	Built

8.5 KPI dashboard

Table 8.5: Nine dashboard indicators

S/N	Identifier	Indicator	Current stored target	Direction	Source
1	OH1	Surveillance compliance	100 per cent	Higher is better	Monthly return
2	OH2	Occupational disease cases	Zero	Lower is better	Occupational plan
3	OH3	Industrial hygiene compliance	At least 95 per cent	Higher is better	Eligible readings
4	OH4	Ergonomic risk control	At least 95 per cent	Higher is better	Actions due
5	S1	Fatality	Zero	Lower is better	Attributed return
6	S2	Total recordable incidents	Zero	Lower is better	Attributed return
7	S3	Total recordable injuries	Zero	Lower is better	Attributed return
8	S4	Reportable near misses	At least 200	Higher is better	Attributed return
9	S5	Health related absenteeism	Under 0.5 days per person	Lower is better	Monthly return plus referral leave

The safety indicator set and directions come from the client dashboard. The current database rows also carry values from the reviewed prototype. They are marked confirmation-pending rather than approved, because review of a prototype did not authorise a threshold or assign its owner.

Two properties of this indicator set are easy to get wrong and are stated as requirements. Fatalities and injuries count people, while recordable incidents and near misses count events, because one event injuring three people is one incident and three injuries. Reportable near misses is the one indicator where a rising number is good, because more near miss reports mean people are reporting, and a dashboard that appears to punish reporting will be given silence.

Table 8.6: Dashboard requirements

S/N	Reference	Requirement
1	DSH-01	Every indicator is shown with its value for the month, its year-to-date value, its target and its status
2	DSH-02	Every indicator states its provenance
3	DSH-03	An indicator with no underlying data reads no data, and never zero
4	DSH-04	No data is presented as a distinct state and never styled as a failure
5	DSH-05	The direction of each indicator is labelled, including where a rising number is good
6	DSH-06	A target whose owner has not approved it is labelled proposed
7	DSH-07	No status is conveyed by colour alone
8	DSH-08	Figures derived from clinical records are aggregates only and never identify a patient
9	DSH-09	A complete year-to-date history totals count indicators and recomputes rate indicators from summed numerators and denominators. An incomplete history shows no figure
10	DSH-10	The selected month leads with off-target and approaching exceptions, while missing data is counted separately as entry outstanding
11	DSH-11	Each indicator shows its monthly trend from January, and a missing month breaks the trend rather than being interpolated

Requirement DSH-03 is the reason the system exists. A dashboard that shows zero where it means it does not know is worse than one that shows nothing, because the reader acts on it.

Requirement DSH-09 records the implementation, but it is not yet a settled client requirement. The current requirements draft calls every year-to-date cell a monthly average. The client-reviewed prototype totals OH2 and S1 to S4, then recomputes OH1, OH3, OH4 and S5 from summed inputs. The implementation follows the prototype because counts accumulate and rates should preserve their denominators. These are two client-facing positions with different meanings, so the Health and Wellness division must resolve DEC-032 before either wording is treated as approved.

9 Non-functional requirements

Table 9.1: Non-functional requirements

S/N	Area	Requirement
1	Security	All traffic encrypted in transit. The clinical database encrypted at rest with a key distinct from other data
2	Privacy	Clinical data is special personal data and does not leave KMC controlled infrastructure

S/N	Area	Requirement
3	Privacy	Non production environments contain synthetic data only and never a real clinical record
4	Privacy	Ordinary request and panic logs record a route template rather than a concrete clinical path, so record identifiers do not enter service logs
5	Availability	Loss of the system must not prevent clinical treatment. A printable form and retrospective entry are available
6	Recovery	Nightly encrypted backup, with a restore tested and recorded quarterly
7	Usability	A user familiar with the paper form completes a routine visit after one orientation session
8	Usability	An error message states the corrective action rather than only reporting failure
9	Accessibility	The interface is operable by keyboard, with visible focus and labelled fields
10	Maintainability	Limits, targets and reference data are changeable without a code release
11	Maintainability	Database changes are applied through versioned migrations held in source control
12	Auditability	Every reported figure is traceable to the records or the return that produced it
13	Branding	The identity provider sign-in pages and the signed-in shell carry KMC branding, while the final product name remains open at DEC-028

10 Decisions required

The following decisions remain open in the stakeholder decision register. The current posture is stated because it explains the implementation, but it is not client approval. Confirmed decisions DEC-018 to DEC-023, DEC-030 and DEC-031 are reflected in the requirements above and are not repeated as approval requests.

Table 10.1: Open decisions and current posture

S/N	Reference	Decision required	Current posture	Owner
1	DEC-024	May management authorisers see individual referral content	No management route exists. The minimum-disclosure summary is officer-only	Accountable sponsor and Data Protection Officer
2	DEC-025	Should the referral form section numbering be corrected	The duplicate Section E labels and missing Section D are reproduced and flagged	Document Control and Health and Wellness Officer
3	DEC-026	Should the printed infirmary officer title be replaced	The printed wording is retained pending a controlled form revision	Document Control and Health and Wellness Officer
4	DEC-027	Who owns the four Workplace Safety figures and their authoritative system	Optional attributed monthly figures are accepted. No incident register is built	Benard Okanyakure, Workplace Safety and KMC ICT
5	DEC-028	What is the final product name	The signed-in shell currently says QHSE Management System and does not mark the name as proposed	Benard Okanyakure and accountable executive sponsor
6	DEC-029	Does the laboratory require structured units, ranges and abnormality flags beyond the supplied form	Results remain free text with no automatic abnormality flag	Laboratory Lead and Health and Wellness Officer

S/N	Reference	Decision required	Current posture	Owner
7	DEC-032	What does each year-to-date value mean	Complete counts are totalled and rates are recomputed from summed inputs. Incomplete histories show no figure	Health and Wellness division, named owner unknown
8	DEC-033	Which month receives referral leave spanning a month end	The full value is assigned to the month in which leave begins	Health and Wellness division and Human Resources, named owner unknown
9	DEC-034	How clinical authenticates to the metrics ingest route	A shared secret is used on an internal route. A scoped service account is required before production	KMC ICT and information security owner, named individual unknown
10	DEC-035	Who may change KPI targets and hygiene limits	The manager may write through the API. There is no reference administration screen	Accountable sponsor and KMC ICT, named business owner unknown
11	DEC-036	Where and by whom the monthly occupational plan is recorded	The officer enters it on Monthly returns and the manager reads it. Confirmed disease placement remains to be agreed	Health and Wellness Manager and Officer
12	DEC-037	Must a patient history download create a distinct audit event	The browser download creates no event beyond the earlier detail read	Data Protection Officer and KMC ICT

10.1 Approval

Approval of this specification means that the requirements stated here are agreed as the basis for build and for acceptance. It does not authorise the processing of real clinical records, which is requested separately.

Table 10.2: Specification sign-off

Role	Name	Signature	Date
Prepared by, Software Engineering Intern			
Reviewed by, Health and Wellness Officer			
Reviewed by, Health and Wellness Manager			
Approved by, Head of Department, Quality, Health, Safety and Environment			